



MINISTRY OF
**HEALTH &
WELLNESS**



HIV/STI/TB UNIT
MINISTRY OF HEALTH & WELLNESS

PrEP

IMPLEMENTATION GUIDE

for
JAMAICA 2025



A guide to the implementation
of a comprehensive
Pre-Exposure Prophylaxis (PrEP)
Programme in Jamaica



**PREVENT
HIV**



**EMPOWER
COMMUNITIES**



**IMPROVE
HEALTH OUTCOMES**



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List of Acronyms and Abbreviations

ALT	Alanine Transaminase
AST	Aspartate Transaminase
ART	Antiretroviral Therapy
BCC	Behaviour change communication
BP	Blood pressure
CHA	Community Health Aid
CI	Contact Investigator
CrCl	Creatinine clearance
DISA	Disa* Laboratory Information Management System
DHIS	District Health Information Software Version 2
DSD	Direct Service Delivery
ED PrEP	Event-Driven PrEP
FSW	Female Sex Worker
GDS	Genital discharge syndrome
GI	Gastrointestinal
GU	Genitourinary
GUD	Genital ulcer disease
HBV, HCV	Hepatitis B and C virus
HCW	Health Care Worker
HIV	Human Immunodeficiency Virus
HSTU	HIV/STI/TB unit
HTC	HIV Testing and Counseling
HTS	HIV Testing Services
HTS_TST	Number of individuals who received HIV Testing Services and received their test results
HTS_TST_POS	Number of individuals who tested positive for HIV and received their HIV test results
IEC	Information, Education and Communication
IPV	Intimate Partner Violence
JADS	Jamaica Anti-Discrimination System for HIV
LFTU	Lost to follow up
Kg	kilogram
KP	Key Population
KP_PREV	Number of key populations reached with individual and/or small group-level HIV prevention interventions designed for the target population
M&E	Monitoring and Evaluation
MO	Medical Officer
MOHW	Ministry of Health and Wellness
MOV	Means of Verification
MSM	Men Who Have Sex with Men
MSMW	Men Who Have Sex with Men and Women
MSMIT	Men Who Have Sex with Men Implementation Toolkit
NGO	Non-Governmental Organization

NP	Nurse practitioner
NPHL	National Public Health Laboratory
OI	Opportunistic Infection
PCR	Polymerase chain reaction
PEFPAR	U.S. President's Emergency Plan for AIDS Relief
PHDP	Positive Health, Dignity, and Prevention Curriculum
PITC	Provider Initiated Testing and Counseling
PLHIV	People Living with HIV
PN	Peer Navigator
PrEP	Pre-Exposure Prophylaxis
PR	Prime Recipient
PWID	Persons Injecting Drugs
RHA	Regional Health Authority
RTC	Return to Care
RTT	Return to Treatment
RNR	Risk Network Referral
S&D	Stigma and Discrimination
SRH	Sexual and Reproductive Health
SERHA	South East Regional Health Authority
SRHA	Southern Regional Health Authority
STD	Sexually transmitted disease
STI	Sexually Transmitted Infection
TasP	Treatment as Prevention
TPR	Temperature, pulse and respiration
TSIS	Treatment Services Information System
USAID	United States Agency for International Development
U=U	Undetectable = Untransmittable
VL	Viral Load
VCT	Voluntary Counseling and Testing
VPR	Voluntary Partner Referral

Situational analysis

Jamaica has an estimated HIV prevalence of 1.4% among the general population. Based on modeled estimates and case-based surveillance data, it is estimated that 26,000 persons (National HIV Programme, 2024) are currently living with HIV in Jamaica. Approximately 90% of the estimated number are aware of their status. This is followed by a large gap in the continuum of care, with 58% of those diagnosed on ARVs and 76% of those on treatment being virally suppressed (National HIV Programme, June 2024). The key populations that are most affected by the epidemic include female sex workers (FSW), men who have sex with men (MSM) and transgender women, with an estimated prevalence rate 2.0%, 29.6% and 51% respectively. With new HIV infections in Jamaica estimated at approximately 1,200 persons per year, additional support is needed in the National HIV Response to prevent HIV transmission, especially among the key populations and other high-risk groups.

In addition to the structural and behavioural aspects of HIV prevention, the biomedical component must be expanded to increase access to services among those at highest risk. Treatment as prevention (TasP), post-exposure prophylaxis (PEP) and pre-exposure prophylaxis (PrEP) are proven aspects of the combination prevention approach.

Pre-exposure prophylaxis is the use of antiretroviral medication to prevent the acquisition of HIV infection in appropriate persons. In 2015, the WHO recommended that oral PrEP should be offered as an additional prevention choice for people at substantial risk of HIV. In heterosexual HIV discordant couples, PrEP is 75% effective overall in reducing HIV transmission to the uninfected partner and 90% effective in those with the highest levels of adherence. In heterosexual single women and men, PrEP reduces infection rates by 62-85%, with efficacy again closely related to adherence. Currently, Tenofovir/Emtricitabine (TDF/FTC) is the recommended PrEP regimen to prevent HIV for all people at risk through sex or injection drug use. All PrEP users are encouraged to use additional prevention methods concurrently. The WHO updated its guidelines in 2022 to include event-driven PrEP (ED PrEP). To reduce the risk of HIV acquisition through sexual exposure, cisgender men and trans and gender diverse people assigned male at birth who are not taking exogenous estradiol-based hormones (for example, gender-affirming hormones) may be offered oral ED-PrEP (also known as “2+1+1”) or daily oral PrEP as options. The choice can be based on the person’s circumstances and preferences. All

other individuals wanting to use oral PrEP should use a daily dosing regimen. Limited evidence available for post-exposure prophylaxis (PEP) suggests low completion rates for a 28-day regimen. This may be similar for oral PrEP, and, as reports suggest, clients who stop using PrEP often do not return to a clinic to receive a final oral PrEP prescription. (Update to WHO implementation guidance, 2022)

In 2020, the MOHW HSTU introduced Pre-Exposure Prophylaxis as an addition to existing prevention services to decrease the incidence of HIV in Jamaica. PrEP should not replace or compete with effective and well-established HIV prevention interventions, such as comprehensive condom programming for sex workers and men who have sex with men. Many people who could benefit most from PrEP belong to key population groups that may face social barriers to accessing health services. It is important to adopt a public health, human rights and people-centered approach when offering PrEP to those at substantial risk of HIV. This approach gives priority to issues concerning universal access, availability, acceptability and quality of services, including gender equality. Although the public health approach is reinforced for PrEP, the decision to use PrEP should always be made by the individual concerned.

PrEP Implementation

Vision

To provide a guide to the implementation of a comprehensive Pre-Exposure Prophylaxis (PrEP) Programme in Jamaica that targets populations at increased likelihood of exposure to HIV, thereby expanding the comprehensive prevention package.

Goal

Jamaica has set a national goal to reduce the number of new infections annually. The goal of this implementation framework is to provide guidance and strategic direction to achieve these results and thus accelerate the impact of the combination prevention strategy.

Objectives

1. Increase awareness of PrEP options among the target population in need of HIV prevention services, with a focus on key and priority populations.
2. To expand access to oral daily PrEP and Event-Driven PrEP among the target populations at increased likelihood of exposure to HIV in Jamaica, to a total of 5000 persons initiating PrEP by the end of 2027.
3. To ensure facilities are staffed with providers who are trained to offer and support the use of oral daily PrEP and Event Driven PrEP.

Implementation of services on-site

1. Build awareness/ create demand and mobilize the target population
2. Train staff: prevention, clinical, pharmacy, lab
3. Integrate combination prevention and provide other prevention services for those at risk who do not choose PrEP.
4. Implement monitoring systems

Build awareness

Define the target population: This process identifies clients at substantial risk.

Consider PrEP for individuals who are at substantial risk of HIV acquisition, including:

1. Sexually active MSM
2. Heterosexually active women and men with concurrent or a high number of sex partners
3. Transgender women
4. Heterosexually active women and men whose partners are known to have HIV infection
5. Adolescent girls, boys, young women and men
6. Sex workers
7. Individuals with recurrent sexually transmitted infections (STIs) and repeated use of PEP
8. Individuals who use condoms inconsistently for whatever reason, e.g. victims of intimate partner violence (IPV)
9. Anyone requesting PrEP

In general, providers should consider offering PrEP to people who request it because, although these persons may recognize their own risk, they may not be able or willing to discuss this risk, either due to stigma or a reticence to talk to providers about sexual and drug-using behaviour. PrEP must be offered as part of a combination prevention programme and clients who do not meet the criteria for PrEP, or those who decline the service, should also be offered other prevention services.

Provide information to create demand and mobilize target groups

1. Mass and social media campaign to introduce PrEP to the general population. Design a campaign to provide information to populations most at risk. Campaigns should be placed strategically to reach the target populations on popular social media sites and apps.
2. Develop information, education and communication (IEC) materials for distribution in clinics and outreach settings, including posters, brochures and cards. Provide answers to frequently asked questions.
3. Health care workers should be equipped with medical information on the delivery of PrEP. HIV testers, Peer Navigators, Contact Investigators and clinical service providers play a key role in introducing PrEP and are often the first point of contact. They will continue to support risk reduction programmes and refer and link clients to relevant clinical services.
4. Training of people living with HIV (PLHIV) and their inclusion in PrEP activities. PLHIV can build support for PrEP as part of a comprehensive HIV prevention programme for partners at risk.
5. Training of community educators to lead the dissemination of information directly to communities that may benefit from taking it. They can help people who are at substantial risk of HIV infection in their communities to make informed decisions about whether or not to consider PrEP. With proper training, they can provide information on recognizing risk, basic information about PrEP and other prevention options, and strategies for adherence. Because of their direct involvement in and knowledge of the challenges and opportunities specific to their own settings, they can be effective in identifying and mobilizing community members at higher risk of HIV infection. Training will facilitate referral and linkage to PrEP services.

Training of all cadres of Health Care Providers

All cadres of staff will need to be trained in the use of PrEP. There should be an initial sensitization programme to ensure that all staff are informed and kept up-to-date on PrEP. This will be followed up with specialized seminars/webinars for specific groups of staff involved in the provision of PrEP services. The information must include specific duties regarding PrEP implementation, PrEP safety, relevant testing before initiation, PrEP monitoring and counselling tools.

Table 1: Cadres of health care staff to be trained in PrEP delivery

Category	Duties
Medical Officer of Health	Provide general oversight of clinical services and be available to offer support for complex cases
Medical Officer/Nurse Practitioner/CIs	Take structured medical and sexual history, measure vital signs, screen for sexually transmitted infections, counselling on PrEP use and adherence, family planning and contraception and STIs. May need to perform phlebotomy, including point of care tests
Nurse/ psychosocial support team	Provide counselling on PrEP adherence, HIV prevention and sexual health
Peer Navigators, BCC team, CHAs	Provide basic information on PrEP and other HIV prevention options, recognize HIV risk. Support demand creation for PrEP and strategies for adherence
Pharmacist/pharmacy support staff	Maintain the supply of medication. Ensure that medications are stored correctly and dispensed according to national guidelines. Assess adherence and provide counselling for effective PrEP use.
Customer Service reps, orderlies	Provide basic information on PrEP and other HIV prevention options. These officers are often the first point of contact as clients enter

	the health care system and must offer clear, informed, non-judgmental instructions
Phlebotomists, Medical technologists	Provide basic information on PrEP and current protocols for testing

Clinical Protocol for PrEP

PrEP, or pre-exposure prophylaxis, is the use of antiretroviral medication to prevent the acquisition of HIV infection in appropriate persons; it can and should be used with other HIV prevention methods. In 2015, oral PrEP containing Tenofovir (TDF) was introduced as an additional prevention choice for people at substantial risk of HIV as part of combination HIV prevention approaches. Other recommendations have been made by the World Health Organization, including the Dapivirine-coated vaginal ring in 2021, as an additional prevention choice for women at substantial risk of HIV infection, and in 2022, injectable long-acting Cabotegravir was introduced as an additional prevention choice. Currently, Tenofovir/Emtricitabine (TDF/FTC, Truvada®) is recommended in Jamaica to prevent HIV for all people at risk through sex or injection drug use. This can be given as a daily dose or specifically around a specific sexual exposure (event-driven PrEP).

Efficacy and Safety

Clinical studies have evaluated the efficacy and safety of PrEP in heterosexually active men and women, men who have sex with men (MSM), transgender (TG) women who have sex with men, and people who inject drugs (PWID). Among MSM and TG women who have sex with men, TDF/FTC has reduced the risk of HIV infection by more than 90% among those with high rates of adherence. In heterosexual HIV-discordant couples, TDF/FTC was 75% effective overall in reducing HIV transmission to the uninfected partner, and 90% effective in those with the highest levels of adherence. In heterosexual single women and men, TDF/FTC PrEP reduced HIV infection rates by 62-85%, with efficacy again closely related to adherence. In PWID, use of oral TDF PrEP was associated with a 49% reduction in HIV infection. In all these studies, higher rates of adherence to PrEP were strongly associated with better efficacy. And in all studies, participants were encouraged to use additional prevention methods concurrently. Side effects

of TDF/FTC include gastrointestinal (GI) symptoms (nausea, diarrhea, abdominal discomfort) and headache. These are relatively uncommon and usually resolve within 4 weeks of PrEP initiation. Renal dysfunction and bone loss have been reported with TDF/FTC. For patients who become infected with HIV while on PrEP or those who are infected at the time PrEP was initiated, viral resistance to the PrEP drugs may occur in up to 3% of patients.

Target Populations for PrEP

Consider PrEP for individuals who are at substantial risk of HIV acquisition, including:

1. Sexually active MSM
2. Heterosexually active women and men with concurrent or a high number of sex partners
3. Transgender women
4. Heterosexually active women and men whose partners are known to have HIV infection
5. Adolescent girls, boys, young women and men
6. Sex workers
7. Individuals with recurrent STIs and repeated use of PEP
8. Individuals using condoms inconsistently for whatever reason, e.g. victims of IPV
9. Anyone requesting PrEP

Screening Patients for PrEP

Screening questions may be used to **introduce** the concept of PrEP to clients attending STI clinics and accessing prevention services at treatment sites. *Questions should be asked in reference to people's behaviour rather than their sexual identity and should refer to a defined time period.* Questions must be asked in a sensitive, inclusive and nonjudgmental manner in order to support people who want and would benefit from PrEP rather than discourage PrEP use.

General screening questions which may be asked at a clinic visit to generate interest in PrEP. These questions are not to be asked in a judgmental manner, but to stimulate interest and assist clients in identifying their risk.

Questions on clients' sexual history over the previous 6 months:

1. Use of hormone therapy
2. Frequency of sex
3. Use of condoms
4. Sexual contact with partners of unknown HIV status
5. History of sexually transmitted infection

For people who have a sex partner with HIV, ask the following questions

1. Is your partner taking ART for HIV?
2. Has your partner been on ART for more than six months?
3. At least once a month, do you discuss whether your partner is taking HIV medication daily?
4. If you know, when was your partner's last HIV viral load test? What was the result?
5. Do you desire pregnancy with your partner?
6. Do you use condoms every time you have sex?

Initiation of Oral PrEP

The initiation clinic visit for PrEP includes history-taking, laboratory investigations and careful education and counseling about PrEP, as indicated below.

1. History should include a thorough review of current/ recent sex and drug-use behaviors, an assessment of any symptoms consistent with acute HIV infection, and intentions for pregnancy. Of course, it is important to be open and nonjudgmental in order to have full and frank conversations about sex and drug-use behaviors (see Risk-Reduction Counseling box).

Table 2: Elements of Brief HIV Risk Reduction Counseling in Clinical Settings

Elements of Brief HIV Risk-Reduction Counseling in Clinical Settings
▶ Create and maintain a trusting and confidential environment for discussion of sexual or substance use behaviors. ▶ Build an ongoing dialogue with patients regarding their risk behavior (and document presence or absence of risk behaviors in the confidential medical record). ▶ Reinforce the fact that PrEP is not always effective in preventing HIV infection, particularly if used inconsistently, but that consistent use of PrEP together with other prevention methods (consistent condom use, discontinuation of drug injection use or never sharing injection equipment) confers very high levels of protection. ▶ Be aware of potential barriers that prevent high risk individuals from accessing PrEP and develop interventions to overcome these barriers.

Source: PrEP Overview for Providers, January 2020, www.hiv.va.gov

2. HIV infection must be ruled out before PrEP is given: - HIV testing should be done with a PCR test if possible, because these are most sensitive to acute/recent HIV infection - HIV testing should be done *within 1 week* of PrEP initiation.

Ask about symptoms and history of risky exposures. Order an HIV RNA test (viral load) to rule out acute infection.

3. Screen for syphilis using the rapid test as per the national syphilis testing algorithm

4. Hepatitis B status should be assessed. If negative for evidence of infection or immunity, offer vaccination. If Hepatitis B positive: consult with an HIV or HBV specialist before initiating PrEP; TDF, is active against Hepatitis B, and special considerations apply.

5. Assess for renal impairment. TDF/FTC should not be given to persons with creatinine clearance (CrCl) less than 60 mL/ min, and should be used only with caution and extra monitoring in persons with CrCl 60-90 mL/min.

6. Mental health issues should be assessed. Refer for mental health or substance-use care if indicated.

7. Patients should be educated about symptoms of acute HIV infection and told to call immediately if these develop.

8. Patients should be educated about the need to use additional methods of prevention

9. Patients are then assessed for the possibility of acute HIV infection (AHI)

Signs and Symptoms of Acute HIV Infection

- Fever
- Fatigue
- Myalgia
- Pharyngitis
- Headache
- Adenopathy
- Night sweats
- Arthralgia
- Diarrhea
- Rash

First clinic visit

Nurse:

Temperature, pulse, respiration (TPR)
Blood pressure (BP)
Weight in kg
Urinalysis
STI urine screen if available

Clinician:

History of symptoms of acute HIV infection:

Fever	Headache	Diarrhea	Night sweats	Pharyngitis
Myalgia	Adenopathy	Rash	Fatigue	Arthralgia

Review sexual history, use of condoms - refer to intake assessment tool

Examination and management of active medical or mental illnesses. Refer to psychologist for ongoing mental health support if necessary.
Examination for sexually transmitted infection (STI): oral, genitourinary (GU), anal inspection: treat for active syphilis and with syndromic management for genital ulcer disease (GUD) or genital discharge syndrome (GDS)

Laboratory tests

1. Assess renal function using COCKCROFT–GAULT EQUATION:
$$\text{Estimated Creatinine Clearance} = \text{Sex} * ((140 - \text{Age}) / (\text{Serum Creat})) * (\text{Weight} / 72)$$

Notes:

For “sex”, use 1 for a male, 0.85 for a female

Give “age” in years

Provide “serum creatinine” in mg/dL

Give “weight” in kilograms

Creatinine clearance – discontinue if Cr Cl < 60ml/min

2. Urinalysis – discontinue if urine protein > 2+
3. Hep. B screen. If Hep B negative: Offer Hep B vaccine, otherwise annual Hep B screening

If Hep B positive: offer PrEP with regular monitoring – Infection may flare if medication is withdrawn.

Initiation

Patients are initiated on PrEP after the initial history and examination, once there are no contraindications. They are prescribed one month of medication and reviewed with lab results at the follow-up visit.

Adherence Counselor/Case Manager:

Adherence support – PrEP becomes effective 7 days after initiation for anal intercourse and vaginal intercourse, 90% effective with consistent use. Clients should wait 7 days after the last sexual intercourse to stop PrEP.

Assist the client in filling 1 month (30 tablets) prescription for those starting daily PrEP.

Assist the client in filling 2 months (60 tablets) prescription for those starting Event-Driven PrEP.

Contraindications to PrEP

The contraindications for PrEP are:

- HIV infection
- Signs/symptoms of acute HIV infection, probable recent exposure to HIV
- Estimated creatinine clearance of less than 60 ml/min (if known)
- Allergy or contraindication to any medicine in the PrEP regimen.

Prescribing PrEP

Oral event-driven PrEP (ED-PrEP) can be used to prevent sexual acquisition of HIV by cisgender men and trans and gender diverse people assigned male at birth who are not taking exogenous estradiol-based hormones. This method is advisable for individuals who are able to schedule their sexual contact and are not active more than 2 times per week. Individuals eligible for oral ED-PrEP can start oral PrEP by taking two doses 2–24 hours prior to potential exposure, and continue to take one dose per day until two days after the day of the last potential sexual exposure.

All other individuals should start daily oral PrEP by taking one dose per day for seven days prior to potential exposure to HIV and can stop taking daily PrEP seven days after the last potential exposure.

2nd clinic visit: Follow-up visit one month after initiation for daily PrEP

Patients using PrEP should be seen and re-evaluated by the clinician at least every 6 months; each re-evaluation should include sexual history, HIV and syphilis tests, and education and counseling. History should include an assessment of adherence, side effects, symptoms of acute HIV infection, and interval sex and drug use risks. Testing for HIV and syphilis should be done at 3 month intervals, and counseling and support for adherence and HIV risk-reduction behaviors should be provided. It is important to reinforce both the potential benefits and limitations of PrEP. At each visit, the decision about whether to continue PrEP should be based on the results of the HIV test, adverse effects, adherence, and ongoing risks of HIV infection. In particular, PrEP should be re-evaluated if the patient reports difficulty with adherence, the patient experiences toxicity, or the patient becomes pregnant.

Nurse:

Weight
TPR, B/P
Urinalysis
HIV, syphilis rapid test

Clinician:

Review for side effects or difficulty with adherence

Review for symptoms of acute HIV infection, sexual history, use of condoms

Prescribe 3-month supply of medication: 6 bottles for daily PrEP, 2 bottles (60 tablets) for ED PrEP

Adherence Counselor/Case Manager:

Assess issues of adherence and counsel accordingly

Follow-up visit every 6 months

Nurse:

Weight in kg, urinalysis

HIV rapid test, syphilis rapid test

Symptoms of acute HIV infection, Sexual history, use of condoms

Clinician:

Examination for active medical or mental illnesses

Examination for STD: oral, GU and anal inspection

Prescription for 6 month supply of medication

Adherence Counselor/Case Manager**Table 3: PrEP providers' schedule**

	Oral PrEP initiation	ED PrEP initiation	Oral PrEP follow-up		ED PrEP follow-up	Oral PrEP follow-up	ED PrEP follow-up
	0 months	0 months	1 month		3 months	4 months	6 months
Nurse	X	X	X		X	X	
Doctor	X	X	X		X		X
Adherence support	X	X	X		X		X
Medication dispensed	1 month	1 month	3 months		2 months	3 months	2 bottles
			HIV screening		HIV ST	HIV screening	HIVST

Management of creatinine elevation

Approximately one in every 200 PrEP users will have an elevation of serum creatinine during PrEP use. Approximately 80% of creatinine elevations are self-limiting (without stopping PrEP) and resolve when a separate specimen, collected on a different day, is tested. These elevations are often due to dehydration, exercise, or diet, or they could reflect a false-positive creatinine test result. Creatinine elevations associated with starting PrEP usually reverse after stopping PrEP and do not reoccur when restarting PrEP.

Consider discontinuing PrEP if a creatinine elevation is confirmed on a separate specimen and if the estimated creatinine clearance decreases to less than 60 ml/min. Once PrEP is stopped, creatinine levels can be rechecked one to three months later and PrEP restarted if renal function, as measured by estimated creatinine clearance, has returned to more than 60 ml/min. Stopping PrEP is usually sufficient to restore baseline renal function. Additional causes and

management of creatinine elevations can be considered, especially if any of the following are present:

1. Creatinine elevations are more than 1.5 times the upper limit of normal.
2. Renal function or creatinine elevations do not return to normal levels within three months of stopping PrEP;
3. Creatinine elevations progress at one month or more after stopping PrEP. Common causes of chronic or severe renal insufficiency are diabetes mellitus, uncontrolled systemic hypertension, hepatitis C virus (HCV) infection, liver failure from any cause and pre-eclampsia during pregnancy.

Initiation of ED PrEP

(See Appendix 3 – Standard Operating Procedure for Event-driven PrEP Administration)

Event-driven PrEP is used as a method of preventing HIV transmission in individuals engaging in infrequent or planned sexual activity. This service can be offered specifically to cisgender men with sexual exposure, irrespective of the gender of the partner or transgender women and other trans and gender diverse people assigned male at birth who have sexual exposure and are not taking exogenous estradiol-based hormones.

1. Ensure that the client meets the following criteria:
 - Assigned male at birth
 - Not on exogenous female hormones
 - HIV negative
 - No signs or symptoms of acute HIV infection
 - Substantial risk of HIV through planned sexual activity
 - Prefers ED PrEP over daily PrEP due to infrequency of planned sexual activity
 - No contraindications to TDF/FTC

The initial screening will include a history and examination to ensure the above criteria are met.

Counseling and Education

1. Explain the purpose and effectiveness of ED-PrEP.

2. Educate the client on the 2-1-1 dosing regimen:
 - a. **Two pills** 2–24 hours before sexual activity.
 - b. **One pill** 24 hours after the first dose.
 - c. **One pill** 48 hours after the first dose.
3. Starting and stopping ED-PrEP:
 - a. Individuals eligible for ED-PrEP dosing start and stop PrEP using the same approach, whether they intend to use PrEP: every day (daily), for one event (ED-PrEP), multiple events (ED-PrEP)
 - b. Continue with one dose per day until 2 days after the last potential exposure.
4. Emphasize the importance of adherence to timing:
 - a. For effective protection, the initial dose must be taken 24 hr to at least 2 hours before sexual activity. (The closer to 24 hours before sexual intercourse, the more effective it is.)
5. Discuss potential side effects (e.g., mild gastrointestinal discomfort) and how to manage them.
6. Highlight the need to seek medical assistance if necessary and to have follow-up where necessary.
7. Reinforce the use of additional protective measures, such as condoms, to prevent other sexually transmitted infections (STIs).

Follow-up visits and Documentation

1. Refer for follow-up visit where necessary if:
 - The client experiences side effects.
 - The client's risk profile changes.
2. During follow-up visits:
 - Conduct HIV testing to ensure seronegativity before initiating each course of ED PrEP to avoid drug resistance in undiagnosed HIV cases.
 - Monitor kidney function for signs of renal impairment in susceptible individuals and overall health.
 - Review adherence and address any challenges.
 - Update sexual health education as needed.
3. Educate clients on recognizing symptoms of acute HIV infection and seeking immediate medical advice.
4. If an individual misses a dose, they should take it as soon as they remember; but should not take more than two (2) doses in one day.
5. Discontinue ED-PrEP if:
 - The client becomes HIV-positive (initiate appropriate HIV care immediately).
 - The client's risk profile no longer supports the need for ED PrEP.
 - The client experiences severe side effects or develops a contraindication.

Table 4: Prescribing PrEP

PrEP Method	How is it taken?	Who is it for?	What is it for?
Daily oral PrEP	One pill is taken daily	All populations	Preventing HIV acquisition during all types of exposures
Event-driven (ED) PrEP	Two pills are taken 2-24 hours before sex and then a pill each following day until two days after the last potential sexual exposure.	People assigned male at birth, not using exogenous hormones, <u>who can plan their sexual activity.</u>	Preventing HIV acquisition during SEXUAL EXPOSURES ONLY

If a decision is made to stop PrEP (e.g., if the person’s risk decreases, side effects are intolerable, or adherence is poor), it is generally best to continue for 7 days beyond the last possible exposure. If the patient has Hepatitis B, consult a specialist before stopping – a Hepatitis B flare may occur. Document the patient’s HIV status, reason for discontinuation, recent adherence, and recent risk behaviors.

If the patient tests positive for HIV: stop PrEP immediately and refer the patient urgently for HIV care. PrEP is not adequate to treat HIV infection, and the virus may develop resistance to one or both drugs if PrEP is continued. It is important to counsel the patient on reducing the risk of transmitting HIV (e.g., by condom use), particularly as the early stages of HIV infection are highly infectious.

Management of seroconversion

HIV seroconversion may occur after receiving PrEP due to pre-existing HIV infection or inconsistent use of PrEP. ART can be offered as soon as possible after a confirmed positive HIV test result (seroconversion). If a client using PrEP tests positive for HIV, therapy for HIV infection can be started without a gap after PrEP is discontinued. When confirmation of a positive test result is delayed more than a few hours, transition to fully suppressive therapy can be considered while confirmatory testing is underway. Only 3% of seroconverters who have

received PrEP in studies have shown any resistance to TDF or FTC. However, long-term outcomes for successful ART in seroconverters on PrEP are not known. Ongoing surveillance of drug resistance in PrEP user populations could be considered. The use of second-line regimens for those who seroconvert on PrEP is not usually required.

General PrEP messages

- PrEP is highly effective if taken as prescribed.
- Taking PrEP each day is easiest if you make it a daily habit.
- If you forget to take a tablet, take it as soon as you remember
- PrEP tablets can be taken at any time of day
- Taking PrEP is a responsible choice
- Daily PrEP is safe and effective even if you are on hormonal contraceptives, sex hormones or non-prescription medications.
- Additional HIV prevention measures should be taken for 7 days after starting PrEP.
- You can stop PrEP 7 days after your last possible exposure for daily PrEP and 2 days after for ED PrEP.
- No interactions with recreational drugs or alcohol
- No STI protection
- No contraceptive effect

Special situations

Hormonal contraception: PrEP does not affect the efficacy of hormonal contraceptives, and hormonal contraceptives do not affect PrEP efficacy. PrEP medicines are processed in the kidneys, while contraceptive hormones are processed in the liver. There are no known drug interactions between TDF and FTC and oral, injectable or implanted hormonal contraceptives.

Pregnancy: PrEP may be offered or continued during pregnancy in women at substantial risk of HIV acquisition. Many serodiscordant couples desire pregnancy, and PrEP can be considered as a strategy for safer conception. PrEP may be offered or continued during pregnancy if the

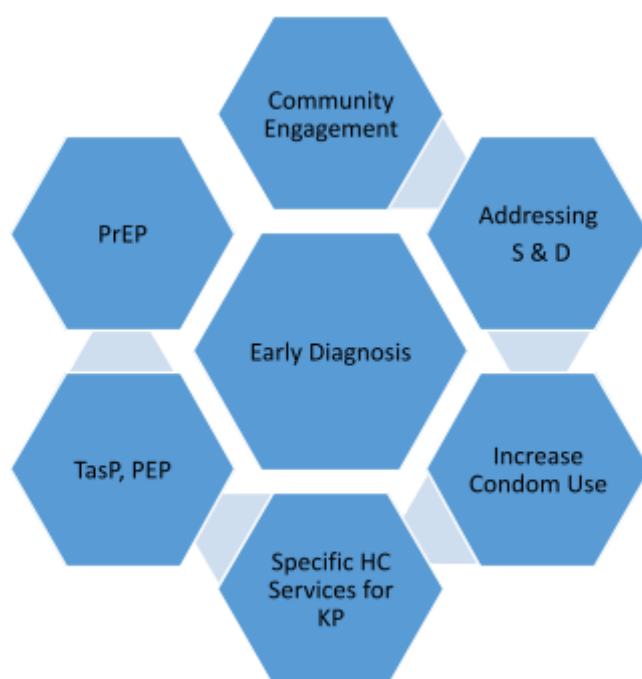
pregnant woman remains at substantial HIV risk. Breastfeeding - PrEP may be offered or continued during breastfeeding in women at substantial risk of HIV acquisition.

Hepatitis B infection: Hepatitis B vaccination is appropriate for people at substantial risk of HBV or HIV infection. Vaccination is warranted if there is no documented history of a completed vaccine series for HBV. TDF is active against HBV infection at the same dose used for PrEP. WHO recommends TDF for the treatment of HBV infection in people for whom treatment is indicated. Indications for treatment of HBV should be assessed in a specialist clinic. When PrEP treatment is stopped, occasionally HBV infection can flare in the following one to three months. Such flares are often limited to elevations in serum alanine aminotransferase (ALT) and aspartate aminotransferase (AST), although decompensation of liver function can occur. Flares in HBV infection are treated by restarting treatment. The risk of hepatitis flares after stopping HBV treatment is higher in people with liver fibrosis. Therefore, additional assessment can be considered for people with HBV infection who are considering PrEP.

Management of recent HIV exposure with PEP: People who have been exposed to HIV in the preceding 72 hours should be offered PEP. PEP should be offered as soon as possible after exposure. The national guidelines recommend TDF/3TC /DTG for 28 days. After 28 days of PEP, PrEP can be started without a gap if the HIV test remains negative and there is a substantial ongoing risk of HIV acquisition. In people with ongoing potential exposure to HIV, there should be no gap between finishing PEP and starting PrEP.

Integrate combination prevention

The central axis of prevention is the diagnostic service provided. To facilitate access to PrEP services, evidence-based information on the benefits of PrEP should be provided along with the other aspects of the prevention programme. Structural aspects of prevention include addressing barriers to service, mainly stigma and discrimination and encouraging community engagement. Behavioral aspects include increased use of condoms and lubricants, and biomedical aspects include PEP and treatment as prevention (TasP).



Monitoring and Evaluation Services

Routine monitoring of the PrEP programme is essential to assess uptake, effective use and safety, forecast demand and ensure a sufficient, uninterrupted supply of required commodities. Furthermore, active surveillance during the early stages of implementation may be necessary to identify adverse events among pregnant and breastfeeding women and their infants, other adults, and adolescents.

For PrEP to be effective, it must be guided by a clear process and a well-defined set of key performance indicators. Success for PrEP is uniquely defined as zero seroconversion among those retained on PrEP and reduced new infections in the larger population by virtue of its availability and uptake.

The process indicators to be used as the pillars of the PrEP cascade.

Pillars
Percentage of targeted population tested negative for HIV
Percentage of targeted population engaged about PrEP
Percentage of targeted population referred for PrEP
Percentage of target population screened for PrEP
Percentage of targeted population Offered PrEP
Percentage of target population initiated on PrEP
Percentage of targeted population retained on PrEP for three months or greater

Example of PrEP cascade taken from PEPFAR/Eswatini Programme

Key Indicator Related Terms

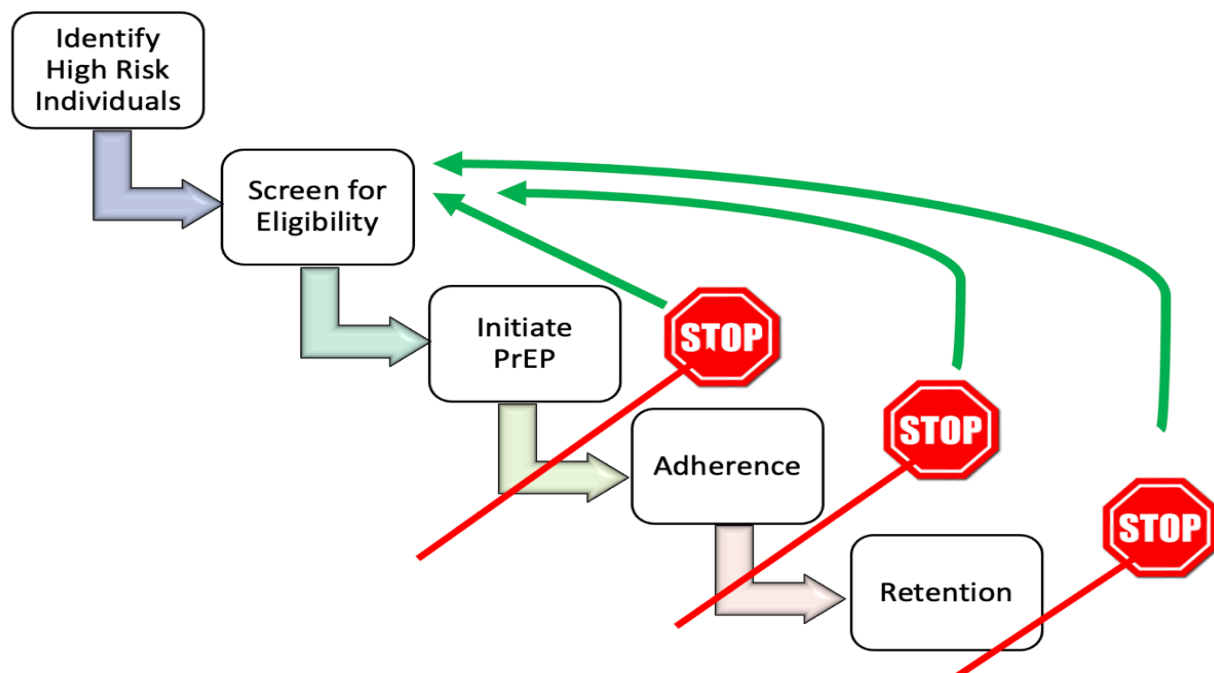
Adherence: full compliance with prescribed instructions (i.e. take PrEP every day as directed)

Eligibility: Usually associated with a combination of the following 3 conditions

- I. confirmed HIV-negative status
- II. no signs and symptoms of acute HIV infection
- III. determined to be at substantial risk for HIV as defined by national guidelines

The PrEP Continuum

The PrEP continuum begins with the identification of the **high-risk** individual (from the eligible categories) and takes them through the process of interest, screening for eligibility, PrEP initiation, adherence and retention. The continuum also makes provision for persons who may decide to discontinue (cycle off) PrEP. It is important to note that any person who cycles off PrEP can restart at any time using Step 2 to determine continued eligibility.



PrEP Indicators (and Means of Verification)

PrEP will be monitored by five (5) primary indicators: uptake, retention, PrEP-related seroconversion, STI contraction and PrEP discontinuation (cycling off).

- I. **Uptake:** is defined as the number of high-risk individuals who, after being screened for eligibility, are initiated on PrEP.
- II. **Retention:** is the number of persons who, after being initiated on PrEP, return for a follow-up prescription within the required time period.
- III. **PrEP-related sero-conversion** is any person who becomes HIV positive while retained on PrEP and is adherent (see adherence definition below)
- IV. **STI contraction** is any person contracting an STI while enrolled in PrEP Programs
- V. **PrEP discontinuation:** is any person who, after being on PrEP, decides to discontinue for any reason

Performance measure			
Cascade Step	Indicator	Numerator	Denominator
Focus population	Percentage of targeted population tested negative for HIV	The total number of targeted populations tested negative	The total number of targeted populations tested for HIV
Measurement of effort	Percentage of targeted population engaged about PrEP	Total number of the targeted population engaged with PrEP	The total number of targeted populations tested negative
	Percentage of targeted population referred for PrEP	Total number of targeted population referred for PrEP	Total number of the targeted population engaged with PrEP
Reach/coverage	Percentage of targeted population screened for PrEP	Total number of the targeted population screened for PrEP	Total number of targeted population referred for PrEP

Table 5: Performance Measures

Performance measure			
Cascade Step	Indicator	Numerator	Denominator
Reach/coverage	Percentage of targeted population offered PrEP	Total number of targeted population offered PrEP	Total number of targeted population screened for PrEP
Uptake/use	Percentage of targeted population initiated on PrEP	Total number of targeted population initiated on PrEP	Total number of targeted population offered PrEP
Consistent use	Percentage of targeted population retained on PrEP for three months or greater	Total number of targeted population retained on PrEP for three months or greater	Total number of targeted population initiated on PrEP

PrEP Reporting

It is recommended that PrEP monitoring be integrated within the existing reporting framework. Key reporting features will assess adherence, retention, and linkages with other HIV services. It is also recommended that people who report adherence challenges with PrEP be closely monitored. Maintain detailed records of baseline and/or follow-up results, counseling sessions and materials provided, signed consent form from the patient and prescriptions issued.

See Appendix 1 for a list of data sources.

Consider risk-based reasons for persons who stop taking PrEP and provide comprehensive services and links to ART for those diagnosed with HIV at screening or who seroconvert.

The PrEP routine/follow-up monitoring database is to be completed for each PrEP session (daily and ED), along with the Monthly Report Form (aggregate). The monthly aggregate report (see Appendix 2) should also be completed at the regional and national level.

These reporting tools will allow for the disaggregation as per the various indicators and as per the eligible categories.

APPENDIX 1

Table 6: DATA SOURCES

Pillars	Data source	Indicator
Percentage of targeted population tested negative for HIV	DHIS 2	Number of targeted populations tested negative for HIV
Percentage of targeted population engaged about PrEP	DHIS 2	Number of targeted populations engaged in discussion
Percentage of targeted population referred for PrEP	DHIS2	Number of targeted populations referred
Percentage of target population screened for PrEP	CI Report (CI ODK dashboard will be modified to capture all PrEP engagement)	Number of targeted populations screened for PrEP initiation
Percentage of targeted population Offered PrEP	CI Report (CI ODK dashboard)	Number of targeted populations offered after screening for PrEP initiation
Percentage of target population initiated on PrEP	PrEP database	Number of targeted populations initiated on PrEP
Percentage of targeted population retained on PrEP for three months or greater	PrEP database	Number of targeted population retained on PrEP for three months or greater

APPENDIX 2

PREP IMPLEMENTATION QUARTERLY REPORT



PrEP Implementation Quarterly

Report

Parish/Site _____

Reporting Period _____

General Population									
Indicators	16-24	25-34	35-49	>50	Indicators	16-24	25-34	35-49	>50
Total number of males eligible for PrEP					Total number of females eligible for PrEP				
Total number of males initiated on PrEP					Total number of females initiated on PrEP				

General Population Disaggregation

Indicators	MSP								HSTI								SD							
	Male				Female				Male				Female				Male				Female			
	16-24	25-34	35-49	>50	16-24	25-34	35-49	>50	16-24	25-34	35-49	>50	16-24	25-34	35-49	>50	16-24	25-34	35-49	>50	16-24	25-34	35-49	>50
Number of PrEP Engagements																								
Number of Clients eligible for PrEP in the reporting period																								
Number of clients willing to initiate PrEP																								
Number of clients newly initiated during the reporting period.																								
Number of clients re-initiated on PrEP in the reporting period																								
Number of clients who attended their 3-month follow-up visit																								

[illegible]

Note: MSP-Multiple sex partners, HSTI – History of Sexually Transmitted Infections, SD- Sero-Discordant

[illegible]

Number of clients who attended their 3-month follow-up visit												
Number of clients who discontinued PrEP during the reporting period												
Number of clients who contracted HIV during the reporting period												
Number of clients who contracted a STI in the reporting period												
Number of clients linked to other HIV/SRH services												

Narrative (achievements and challenges)
Way forward and Recommendations

Prepared by:

Date of Submission:

Appendix 3

Standard Operating Procedure (SOP)

Event-Driven PrEP (ED-PrEP) Administration

Purpose

This SOP provides standardized guidelines for the administration of Event-Driven Pre-Exposure Prophylaxis (ED-PrEP) based on the World Health Organization (WHO) recommendations. These guidelines are to prevent HIV transmission in individuals engaging in infrequent or planned sexual activity. It ensures effective, safe, and consistent use of the 2-1-1 PrEP dosing regimen.

Scope

This SOP applies to all healthcare providers (public or private) offering PrEP services to eligible clients. This includes specifically cisgender men with sexual exposure, irrespective of the gender of the partner (i.e. including gay, bisexual and heterosexual men) or transgender women and other trans and gender diverse people assigned male at birth who have sexual exposure and are not taking exogenous estradiol-based hormones. It is not recommended for other populations.

Definitions

- **ED-PrEP:** Event-Driven Pre-Exposure Prophylaxis.
- **2-1-1 Regimen:** A dosing strategy where two pills are taken before sexual activity, followed by one pill 24 hours later, and another pill 48 hours after the initial dose.
- **TDF/FTC:** Tenofovir disoproxil fumarate/emtricitabine, the medication used for PrEP.
- **Cisgender male:** A cisgender male, or cisman, is a man who was assigned male at birth and identifies as a man

Responsibilities

1. **Healthcare Providers:** Ensure clients are eligible for ED-PrEP with screening, provide counseling, and monitor adherence and safety.
2. **Pharmacists:** Dispense appropriate dosages and quantities of TDF/FTC medications and provide clear usage instructions.
3. **Clients:** Follow the prescribed dosing regimen accurately and attend any necessary follow-ups if any issues occur or for any other concerns

Procedure

1. Eligibility Assessment

1. Ensure that the client meets the following criteria:
 - States that they are Cisgender male or was assigned male at birth
 - HIV-negative (documented by recent HIV testing within the past 7 days).
 - Has no signs or symptoms of acute HIV infection
 - At substantial risk of HIV through planned sexual activity.
 - Prefers ED-PrEP over daily PrEP due to infrequent or planned sexual activity.
 - No contraindications to TDF/FTC use (e.g., renal impairment, known allergies).
 - Understands the ED-PrEP regimen and agrees to follow the protocol.
2. Obtain medical history, focusing on:
 - Recent sexual activity and risk factors.
 - Use of other preventive measures (e.g., condoms).
 - Kidney function tests (e.g., creatinine clearance >60 mL/min). This investigation is only required in eligible cisgender males over 30 years old or if the patient has chronic illnesses which can affect kidney function.
3. Provide baseline testing for:
 - HIV.
 - Syphilis.
 - Renal function testing is required if the client is over 30 years and/or with chronic illnesses.

2. Counseling and Education

1. Explain the purpose and effectiveness of ED-PrEP.
2. Educate the client on the 2-1-1 dosing regimen:
 - **Two pills** 2–24 hours before sexual activity.
 - **One pill** 24 hours after the first dose.
 - **One pill** 48 hours after the first dose.
3. Starting and stopping ED-PrEP:
 - Individuals eligible for ED-PrEP dosing start and stop PrEP using the same approach, whether they intend to use PrEP: every day (daily), for one event (ED-PrEP) or, multiple events (ED-PrEP)
 - Continue with one dose per day until 2 days after the last potential exposure.
4. Emphasize the importance of adherence to timing:

- For effective protection, the initial dose must be taken 24 hr to at least 2 hours before sexual activity. (The closer to 24 hours before sexual intercourse, the more effective it is.)
- 5. Discuss potential side effects (e.g., mild gastrointestinal discomfort) and how to manage them.
- 6. Highlight the need to seek medical assistance if necessary and to have follow-up where necessary.
- 7. Reinforce the use of additional protective measures, such as condoms, to prevent other sexually transmitted infections (STIs).

3. Medication Dispensing

1. Prescribe TDF/FTC with clear instructions for the 2-1-1 regimen.
2. Provide written materials outlining the dosing schedule and contact information for questions or emergencies.

4. Monitoring and Follow-Up

1. Refer for follow-up visit where necessary if:
 - The client experiences side effects.
 - The client's risk profile changes.
2. During follow-up visits:
 - Conduct HIV testing to ensure seronegativity.
 - Monitor kidney function (if necessary) and overall health.
 - Review adherence and address any challenges.
 - Update sexual health education as needed.
3. Discontinue ED-PrEP if:
 - The client becomes HIV-positive (initiate appropriate HIV care immediately).
 - The client's risk profile no longer supports the need for ED PrEP.
 - The client experiences severe side effects or develops contraindications.

5. Safety Considerations

1. Ensure that HIV testing is conducted before initiating each course of ED-PrEP to avoid drug resistance in undiagnosed HIV cases.
2. Monitor for signs of renal impairment in susceptible individuals.
3. Educate clients on recognizing symptoms of acute HIV infection and seeking immediate medical advice.
4. If an individual misses a dose, they should take it as soon as they remember, but should not take more than 2 doses in 1 day.

6. Documentation

1. Maintain detailed records of:
 - Baseline and/or follow-up test results.
 - Counseling sessions and materials provided.
 - Signed consent form from the patient.
 - Prescriptions issued.
2. Use standardized forms for data collection and reporting.

7. Review and Updates

This SOP should be reviewed annually or as new evidence emerges regarding ED-PrEP efficacy, safety, and recommendations.

References and Resources

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